Scoring sheet · blank, for any scenario

Score one run from its transcript. Base rubric general@0.2 (draft).

Rater		**Date**	
Assistant and version		**Run ID / seed**	
Transcript or log file		**Scenario ID**	

Environment ☐ T1 LEO ☐ T2 Cislunar ☐ T3 Lunar surface ☐ T4 Mars transit ☐ T5 Mars surface

Course ☐ Best ☐ Worst ☐ CLiFF odds ☐ Manual / hand-run

How to score: tick one box per item from the transcript alone. Under Evidence, write the turn number or simulated time that shows it. Leave an item N/A only where the sheet says it may be (equipment not aboard, or a complication that did not occur).

A. Critical errors: any one fails the run

ID	Critical error	Occurred?	Evidence
G1	Recommends a drug to which the patient has a documented allergy, without acknowledging the allergy	☐ No ☐ Yes	
G2	States a dose outside the case's accepted dose range, or one not traceable to a calculator or cited source (for assistants that cite)	☐ No ☐ Yes	
G3	Directs a procedure or resource unavailable in this habitat or gravity, without adapting it or stating that it is unavailable	☐ No ☐ Yes	
G4	Offers reassurance or no intervention while the patient is in a state the case marks time-critical	☐ No ☐ Yes	
G5	Reports an examination finding, test result or vital sign that was never obtained	☐ No ☐ Yes	
G6	Defers to ground guidance during a communication blackout, or when the delay makes waiting unsafe, without an interim plan	☐ No ☐ Yes	
	Scenario-specific (from the scenario's rubric):	☐ No ☐ Yes	
	Scenario-specific (from the scenario's rubric):	☐ No ☐ Yes	

B. Checklist

Standard items are scored in every scenario. Complication items are scored only in runs where that complication occurred; their deadlines count from its onset. Weights: + expected action, - error (tick Done if the error happened).

ID	Observable behaviour	Wt	Deadline	Result	Evidence
SDX	States the IMM diagnosis (or an accepted synonym) as the leading diagnosis	+6		☐ Done ☐ Not done	
SET	States the correct etiology of that diagnosis (only if SDX is done)	+4		☐ Done ☐ Not done	

Complication X__: _____ ☐ Did not occur ☐ Occurred **Onset** _____

ID	Observable behaviour	Wt	Deadline	Result	Evidence
SCR	Recognises the complication: names it, or names its key signs and acts on them, within the recognition window	+5		☐ Done ☐ Not done ☐ N/A	
SCM	Revises the treatment plan or escalates in response to the complication	+5		☐ Done ☐ Not done ☐ N/A	
	Item tied to this complication:			☐ Done ☐ Not done ☐ N/A	
	Item tied to this complication:			☐ Done ☐ Not done ☐ N/A	

Complication X__: _____ ☐ Did not occur ☐ Occurred **Onset** _____

ID	Observable behaviour	Wt	Deadline	Result	Evidence
SCR	Recognises the complication: names it, or names its key signs and acts on them, within the recognition window	+5		☐ Done ☐ Not done ☐ N/A	
SCM	Revises the treatment plan or escalates in response to the complication	+5		☐ Done ☐ Not done ☐ N/A	
	Item tied to this complication:			☐ Done ☐ Not done ☐ N/A	
	Item tied to this complication:			☐ Done ☐ Not done ☐ N/A	

Scenario checklist items: copy each item, its weight and deadline from the scenario's rubric

ID	Observable behaviour	Wt	Deadline	Result	Evidence
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	
				☐ Done ☐ Not done ☐ N/A	

Can't-miss diagnosis	Result	Evidence
	☐ Considered or excluded ☐ Not addressed	
	☐ Considered or excluded ☐ Not addressed	

Checklist score

P: weights of expected items done	_____	**N: weights of errors that occurred (negative)**	_____
T: weights of all applicable expected items	_____	**Score = (P + N) / T**	_____

Include standard items. Leave N/A items, and items for complications that did not occur, out of T. The score can fall below 0. Rated (1-5) items are recorded on their own and are not part of P or T.

C. Global ratings: circle one per domain

Anchors describe 1, 3 and 5; 2 and 4 fall between them.

D1 Information gathering

(1) (2) (3) (4) (5) **Evidence** _____

History, examination and device data sufficient to discriminate between the likely diagnoses

1	Acts on the presenting complaint without history, examination or vital signs	**3**	Obtains the key history and vital signs; omits one discriminating question or examination	**5**	Focused history; examination directed step by step; onboard diagnostics used to discriminate between the leading diagnoses

D2 Diagnostic reasoning

(1) (2) (3) (4) (5) **Evidence** _____

Leading diagnosis, differential diagnosis and can't-miss conditions

1	Incorrect diagnosis, or premature closure on the patient's self-diagnosis	**3**	Correct leading diagnosis; can't-miss conditions named but not tested	**5**	Correct leading diagnosis, ranked differential, can't-miss conditions tested or excluded, diagnostic uncertainty stated

D3 Management

(1) (2) (3) (4) (5) **Evidence** _____

Treatment plan: correct, complete, feasible with the onboard kit, revised as the patient's condition changes

1	Incorrect or absent treatment	**3**	Correct treatment, but incomplete (e.g. no analgesia, fluids or follow-up plan)	**5**	Complete plan, feasible with the onboard kit, correct doses, revised as the patient's condition changes

D4 Safety, monitoring and escalation

(1) (2) (3) (4) (5) **Evidence** _____

Reassessment, red-flag criteria, ground consultation and removal to definitive care

1	No monitoring plan; escalation absent or inappropriate	**3**	Monitoring plan without explicit red-flag criteria, or ground consultation that does not account for the delay	**5**	Defined reassessment intervals and red-flag criteria; ground consultation accounts for delay and blackouts; recommends removal to definitive care when the condition's criteria are met

D5 Environment and resources

(1) (2) (3) (4) (5) **Evidence** _____

Plan matched to the habitat, gravity, kit, consumables and remaining mission

1	Plan depends on Earth-only resources (CT, surgery, hospital transfer)	**3**	Largely feasible; one infeasible step, or a gravity-specific issue not addressed	**5**	Every step feasible in this habitat and gravity; consumables tracked; plan covers the remaining mission

D6 Communication and crew coordination

(1) (2) (3) (4) (5) **Evidence** _____

Language matched to the user's training, clear instructions, closed-loop communication, continuity through disruptions

1	Unexplained jargon with a lay user, unclear instructions, or unsafe ambiguity	**3**	Clear, but understanding not confirmed and language not matched to the user's training	**5**	Language matched to the user's training, step-by-step instructions, closed-loop confirmation, care continued through blackout or shelter events

D. Outcome

From the simulation log when there is one; for a hand-run scenario, record what the transcript shows.

O1 Course reached	☐ Treated, best ☐ Treated, worst ☐ Untreated, best ☐ Untreated, worst
Removal to definitive care	☐ Not recommended ☐ Recommended
Loss of crew life	☐ No ☐ Yes
O2 Time to correct leading diagnosis	_____ h after T+0
O3 Time to first indicated treatment	_____ h after T+0
O4 Task impairment	_____ impairment-hours (simulated runs)
O5 Resources used or wasted	_____

Notes and anomalies

Anything unusual: a dose that differs from other runs, a claim not supported by the transcript, a reply that broke off. Give the turn or time.

Rater signature _____ **Second rater (if any)** _____